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Official Constituent Organization of the American Academy of Physician Associates (AAPA)

September 14, 2026

Submitted electronically via www.regulations.gov

Administrator

Centers for Medicare & Medicaid Services

U.S. Department of Health and Human Services

Attention: CMS-1848-P

P.O. Box 8016

Baltimore, MD 21244-8016

**RE: CMS-1848-P — CY 2027 Medicare Physician Fee Schedule Proposed Rule;
Docket CMS-2026-2377**

Dear Administrator:

The Society of Addiction Medicine Physician Associates (SAMPA) appreciates the opportunity to comment on the Calendar Year (CY) 2027 Medicare Physician Fee Schedule (PFS) proposed rule (CMS-1848-P). SAMPA is an official Constituent Organization of the American Academy of Physician Associates (AAPA). SAMPA represents physician associates/physician assistants (PAs) who provide evidence-based prevention, treatment, and recovery services for individuals with substance use disorders (SUD) in every state and practice setting, including rural, office-based, emergency, and telehealth settings.

SAMPA's comments are limited to addiction-care payment and PA practice. We recognize that CMS cannot restore the temporary statutory conversion-factor increase that expires after CY 2026; we do not ask CMS to reverse that statutory change. We ask CMS to finalize the addiction-care payment improvements already in this rule, and to modify a small number of proposals that would otherwise shrink access to SUD care furnished by physicians and qualified nonphysician practitioners, including PAs.

Where we ask CMS to modify or oppose proposed text, we quote the language as proposed and then supply the exact replacement language we want in the final rule.

1. SBIRT and tobacco-cessation work RVUs — SUPPORT (finalize as proposed)

As proposed: In the CY 2024 PFS final rule, we finalized an increase in the valuation for timed behavioral health services by applying an upward adjustment to the work RVUs for psychotherapy codes payable under the PFS. This increase is being implemented over a four-year transition period. We believe similar adjustments are warranted for smoking and tobacco use cessation and screening, brief intervention, and referral to treatment (SBIRT) services. Therefore, for CY 2027, we are proposing to include smoking and tobacco use

cessation services and SBIRT services in this final year of the transition for timed behavioral health codes.

Comment: SAMPA strongly supports this proposal. SBIRT and tobacco-cessation counseling are core tools for identifying unhealthy substance use and engaging patients in care. Physicians and qualified nonphysician practitioners, including PAs, furnish these services in primary care, emergency, hospital, and addiction settings. Underpayment has long discouraged routine screening. A one-year catch-up in the final transition year is the right policy.

Suggested revision: None. Finalize the proposed inclusion of smoking and tobacco use cessation services (CPT 99406, 99407) and SBIRT services (HCPCS G2011, G0396, G0397) in the final year of the timed behavioral health work RVU transition for CY 2027, as proposed.

Future rulemaking (not in this NPRM): In a future PFS proposed rule, apply the same work RVU adjustment to annual alcohol misuse screening and brief counseling codes G0442 and G0443 so the screening family is valued consistently.

2. Shared medical appointments (HCPCS GSMAS) — SUPPORT the code and qualified nonphysician practitioner leadership; MODIFY the eligible-condition list

As proposed (code descriptor): HCPCS code GSMAS: Voluntary, group-based medical session involving multiple patients with common medical condition(s), receiving medical care in a group setting; billed and led by a physician or qualified nonphysician practitioner and may include services provided by other qualified healthcare professionals, clinical staff, or auxiliary personnel under the direction of the supervising physician or other practitioner. Session integrates group education, counseling, and peer support with individualized patient clinical assessment and care, 2-10 patients, billed once per patient, per session.

As proposed (leadership): The sessions are generally led by a physician, physician assistant (PA), or an advanced practice registered nurse (APRN)... We propose that a SMA session is billed and led by a physician or qualified nonphysician practitioner...

As proposed (eligible conditions): SMAs are an appropriate healthcare delivery approach for conditions that are modifiable with lifestyle change, including diabetes mellitus, obesity, hypertension, and hyperlipidemia, such that behavioral changes including diet, physical activity, and self-management can influence health outcomes. Therefore, we propose to establish coding and payment for SMAs provided for medical conditions that are modifiable with lifestyle change.

Comment: SAMPA supports creation of GSMAS and strongly supports the physician or qualified nonphysician practitioner leadership language as written. Tobacco use disorder, alcohol use disorder, and other SUDs are chronic diseases likewise modifiable with lifestyle change, peer support, and self-management. The proposed examples are cardiometabolic. Unless tobacco use disorder, alcohol use disorder, and other substance use disorders are expressly included, SUD-focused SMAs will not clearly qualify.

Suggested revision (eligible conditions): SMAs are an appropriate healthcare delivery approach for conditions that are modifiable with lifestyle change, including diabetes mellitus, obesity, hypertension, hyperlipidemia, tobacco use disorder, alcohol use disorder, and other substance use disorders, such that behavioral changes including diet, physical activity, peer support, and self-management can influence health outcomes. We are finalizing coding and

payment for SMAs provided for medical conditions that are modifiable with lifestyle change, expressly including tobacco use disorder, alcohol use disorder, and other substance use disorders.

Suggested revision (leadership): None. Finalize the GSMAS descriptor and leadership language as proposed (physician or qualified nonphysician practitioner; sessions generally led by a physician, PA, or APRN). Do not narrow the final rule to physician-led only.

3. E/M visit complexity add-on (HCPCS G2211) — SUPPORT percentage form (MOD1); MODIFY ACO-only MOD2

As proposed (MOD1): For CY 2027, we are proposing two changes. We are proposing to transition HCPCS code G2211 to a modifier that can be appended to the associated E/M base code (placeholder modifier MOD1 will be replaced with a two-digit HCPCS modifier if finalized). This modifier would increase the payment of the associated E/M code by 16%, instead of a flat rate, maintaining an equal percentage increase across all levels of E/M codes.

As proposed (MOD2): This modifier (placeholder modifier MOD2) would be available only for practitioners participating in a Shared Savings Program ACO or Participant Providers in a Long-term Enhanced ACO Design (LEAD) Model ACO and would increase payment of the associated E/M visit by 32%.

Comment: SAMPA supports moving from a flat G2211 add-on to a percentage increase (MOD1). Longitudinal addiction care—induction, dose adjustment, relapse prevention, and co-occurring psychiatric and medical management—is precisely the continuous clinician relationship G2211 was designed to support. Creating a substantially larger percentage increase only for ACO / LEAD clinicians would pay independent and small-practice physicians and qualified nonphysician practitioners, including PAs, less for identical longitudinal SUD/MOUD work.

Suggested revision (MOD1): None. Finalize placeholder modifier MOD1 as proposed (16% of the associated E/M).

Suggested addition (MOD1): Ongoing office-based substance use disorder treatment or medications for opioid use disorder management qualifies as a single, serious, or complex condition for purposes of reporting the visit-complexity modifier, and the modifier may be reported by physicians and qualified nonphysician practitioners who furnish that care.

Suggested revision (MOD2): This modifier (placeholder modifier MOD2) would be available to physicians and qualified nonphysician practitioners who meet the visit-complexity criteria, including practitioners furnishing ongoing SUD or MOUD management, and would increase payment of the associated E/M visit by 32%. Participation in a Shared Savings Program ACO or a LEAD Model ACO is not required to report the modifier.

Fallback if CMS will not extend MOD2 eligibility: Do not finalize ACO/LEAD-only eligibility for MOD2; apply only MOD1 (16%) to all practitioners who meet the visit-complexity criteria.

4. Quality ID 305 — OPPOSE removal from the APP Plus quality measure set

As proposed: We are proposing several changes to the APP Plus quality measure set for Shared Savings Program ACOs, including the removal of Initiation and Engagement of Substance Use Disorder Treatment (Quality ID: 305) and Adult Immunization Status (Quality ID: 493) from the APP Plus quality measure set. ... We are proposing that the APP Plus quality measure set for Shared Savings Program ACOs would include eight measures ... beginning with PY 2027.

Comment: Removing Quality ID 305 eliminates a payment incentive to start and retain people in SUD treatment and works against CMS's behavioral-health and chronic-disease objectives. ACO reporting burden should not be solved by dropping the SUD initiation and engagement measure. Physicians and qualified nonphysician practitioners, including PAs, in ACO-participating practices initiate and engage Medicare patients in SUD treatment; removing Quality ID 305 reduces accountability for that care.

Suggested revision: We are proposing several changes to the APP Plus quality measure set for Shared Savings Program ACOs, including the removal of Adult Immunization Status (Quality ID: 493) from the APP Plus quality measure set. We are retaining Initiation and Engagement of Substance Use Disorder Treatment (Quality ID: 305) in the APP Plus quality measure set for Shared Savings Program ACOs beginning with PY 2027 and subsequent performance years.

PAs have been an integral part of the healthcare landscape for nearly 60 years, supporting patients and communities in all settings, including office-based opioid treatment, Rural Health Clinics, Federally Qualified Health Centers, emergency departments, and telehealth. SAMPA stands ready to serve as a resource to CMS on workforce payment, office-based medications for opioid use disorder, screening and brief intervention, and the practical realities of delivering addiction care under the Physician Fee Schedule. We appreciate your consideration of these comments and look forward to continued engagement as this important work proceeds.

Please direct any questions regarding these comments to Natasha Seliski, PA-C, MPAS and Arianna Campbell, DMSc, MPH, PA-C, CAQ-EM, DFAAPA, co-chairs of the SAMPA Public Health Policy Committee at policy@addictionpas.org.

Respectfully submitted,

Shani Wilson, PA-C

President

Society of Addiction Medicine Physician Associates (SAMPA)